SAMPLE ANSWER · CEREBRAL PALSY · CASE 4

Sample 7-minute ISBAR: Ethan

Ethan Clarke · 32-year-old man · Supported independent living home, NDIS-funded · about 1038 spoken words (≈ 7.4 min at 140 words per minute)

I Introduction

62 words

"Hi, I'm the occupational therapy student who has been completing Ethan Clarke's wheelchair seating review at his supported independent living home. Ethan is a 32-year-old man with spastic quadriplegic cerebral palsy, GMFCS level IV. I need your help urgently because Ethan has an open wound over his sacrum, he's in pain, and he has been without his communication device for two weeks."

S Situation

118 words

"During a hoist transfer to his bed, I saw a purple area about the size of a 50-cent coin over the base of his spine, with a small open, weeping centre. Using yes/no, Ethan told me he has pain in his bottom. He's been banging his tray and shifting his weight, which staff describe as aggression.

"His speech-generating device has been broken for about two weeks. A support worker, Jade, has asked me to recommend wrist straps to stop him hitting. A new cushion from a different supplier arrived three weeks ago. His maxi taxi is due in fifteen minutes for a four-hour shift at the radio station, and the house manager wants an armrest repair quote signed."

B Background

144 words

"Ethan lives in NDIS-funded SIL in Ipswich with two housemates. He's GMFCS level IV and MACS level IV, drives a powered wheelchair with a joystick, and usually communicates with a speech-generating device on his tray, using his right hand through a keyguard. Without it, he nods for yes and turns away for no. He works two days a week at a community radio station.

"His seating review 18 months ago documented good posture and intact skin. Progress notes say he's medically stable, and his GP saw him last month for repeat prescriptions. The referral was for a loose armrest and sliding forward.

"There's no documentation of who prescribed or set up the new cushion, his repositioning routine, skin checks, continence care, whether the device repair has been requested, or whether he has a behaviour support plan. Jade said night staff do his personal care."

A Assessment

337 words

"My first concern is the wound. A purple area with an open centre over the sacrum, in someone who sits most of the day and can't easily reposition himself, may indicate a pressure injury, and the depth needs assessing by a nurse or doctor. The timing fits with the new cushion three weeks ago, and his sliding forward may increase pressure and shear over the sacrum. If he sits for a four-hour shift today, the wound is likely to worsen, with a risk of infection.

"Although his skin was intact 18 months ago, that tells us nothing about now. And medically stable on paper hasn't protected him, because nobody appears to have been checking his skin.

"My second concern is that Ethan's behaviour is communication. He's in pain and has lost his main way of telling anyone. Hitting his tray, his usual access point for his device, is probably the most effective signal he has left. Staff saying he's been like this before doesn't mean it's acceptable or that the cause was explored.

"That's why I can't recommend wrist straps. Restraining his wrists would be a mechanical restraint, a regulated restrictive practice that needs authorisation and a behaviour support plan under the NDIS rules and Queensland's restrictive practice framework. More importantly, it would remove the one way he has of telling us something's wrong while he's in pain. I'd treat the request as a safeguarding concern rather than a solution.

"My third concern is communication access. Two weeks without his device affects his rights, choices, work at the radio station, and ability to report pain or mistreatment. Restoring his voice is a priority, not an equipment detail.

"Together, an unnoticed wound, an unrepaired communication device and a request for restraint raise questions about the quality of his support. The provider and you may need to consider whether this meets NDIS Commission reportable incident criteria.

"The armrest quote, the footy and his mum's NDIS questions all matter to Ethan, but they come after the wound and his voice."

R Recommendation

377 words

"My immediate recommendation is that Ethan stays off his sacrum, lying on his side on the bed, rather than sitting for his shift today. I'd explain this to him and ask, using yes/no, whether he agrees and whether he'd like us to let the radio station know. It's his decision, but I'd make sure he understands the risk.

"I'd like your help to arrange same-day wound assessment, through his GP or community nursing, depending on what's available. I'll document what I saw and, with Ethan's consent and per policy, photograph it so it can be monitored.

"I'd decline the wrist strap request, explain why to Jade respectfully, and raise it with the house manager and you. I'd ask that staff record his signs of distress, so the team can understand what he's communicating.

"I'd like us to raise the device with the house manager together. I'd ask that the repair is treated as urgent, and that, in the meantime, Ethan has a low-tech communication board with pain, body-part and yes/no options. I'd contact his speech pathologist or AAC provider today.

"I'd ask you to guide whether any of this must be reported as an incident.

"For the team, I'd ask the GP or nurse to stage and treat the wound and check for infection and pain. I'd ask nursing to set a repositioning schedule and skin-check routine with staff. Speech pathology would restore and review his AAC access. A behaviour support practitioner could be involved if needed, focusing on understanding his communication rather than restricting it.

"For OT, my principal short-term assessment would be interface pressure mapping with the new cushion compared with his prescribed type, alongside a seated posture assessment to look at why he's sliding forward, once the wound team agrees it's safe for him to sit. Longer term, once his device is repaired, I'd use the COPM with Ethan, so he can tell us his own priorities about seating, work and daily life.

> "I'd hold off signing the armrest quote until the full seating review is done. I'd check with Ethan whether he's happy for me to talk to his mum before discussing his plan review.
>
> "I'd follow up tomorrow and arrange to finish the seating review once his skin and communication are addressed."

Coaching notes *not spoken*

The immediate priority

Ethan has a likely pressure injury over his sacrum and is in pain, but has been without his voice for two weeks, so his distress is being labelled as aggression. Keep him off the wound, arrange same-day wound assessment, restore his communication and decline the wrist strap request.

Distractors in this case

- **"Skin intact" 18 months ago and "medically stable" notes** — out of date and not based on skin checks.
- **"Aggressive" and "been like this before"** — behaviour is communication, especially without AAC.
- **Wrist strap request** — an unauthorised restrictive practice and a safeguarding concern, not a seating solution.
- **Armrest quote, footy, mother's NDIS call, taxi to work** — real but secondary; respect Ethan's autonomy about work while explaining the risk.

Missing information to clarify

- Who prescribed and set up the new cushion
- Repositioning, skin-check and continence routines
- Device repair status and funding
- Behaviour support plan, if any
- Ethan's consent to share information with his mother

Useful references

- NDIS (Restrictive Practices and Behaviour Support) Rules 2018
- EPUAP/NPIAP/PPPIA (2019). Prevention and Treatment of Pressure Ulcers/Injuries: Clinical Practice Guideline
- NDIS Quality and Safeguards Commission guidance on reportable incidents

Unofficial practice material. All cases are fictional and written for ISBAR communication practice; they are not clinical guidance. Not produced by or affiliated with any university.